

Q3 2026 Tabletop Packet

CARE INDEED HOME HEALTH CARE, INC.

Q3 2026 — Growth Outruns Control

Census grows from 142 to 170 in a single quarter. Two new PIPs (hospitalization spike, care-coordination gap) surface alongside five improving-but-not-closable carry-forward PIPs, a second documentation irregularity from a reinstated clinician, three new-hire competency matters, and four governance actions the Board must transact the same meeting: a Clinical Manager transition, a scope-of-services change, a budget authorization tied to CAP sustainability, and a media inquiry into the quarter's fall pattern.

FABLE GENERATED REVIEW REQUIRED SYNTHETIC MOCK DATA - NO REAL PHI - NOT FOR PRODUCTION

PERIOD	SOURCE CUTOFF
Q3 2026	2026-10-08
PACKET ID	VERSION
CI-GB-TT-Q3-2026-V1-0-2	1.0.2
CASE-PACK ID	PASS MODEL
GB-TT-Q3-2026	950 / 1000 plus critical-error rules

SYNTHETIC MOCK DATA - NO REAL PHI - NOT FOR PRODUCTION

All records in this packet are approved synthetic case-pack data. This packet is not production evidence and contains no legal signature, vote, quorum determination, or completed action beyond what the normalized case pack supports.

FRONT MATTER

Document Control and Use Conditions

GENERATED BY

Fable deterministic packet generator

CLASSIFICATION

SYNTHETIC MOCK DATA - NO REAL PHI - NOT FOR PRODUCTION

SOURCE CUTOFF

2026-10-08

ARTIFACT STATUS

Review required

SOURCE MODEL

GB-TT-Q3-2026

RELIANCE

Not approved for UAT until human visual and evidentiary review is complete

Use limitation

The Q1 exemplar supplied by the user informed typography, color, hierarchy, and table treatment only. No fictional name, metric, signature, quorum, vote, approval, or lock-ready claim from that exemplar is a factual source for this packet.

Evidence integrity

Unresolved values remain unresolved.
Supplemental UAT records remain labeled. Post-cutoff items do not rewrite historical measurement posture.

Approval boundary

Viewing this PDF does not approve the case, satisfy readiness, create evidence, or complete a tabletop attempt.

FRONT MATTER

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FRONT MATTER

How to Read This Packet

- Evidence first.**

Review source posture, cutoff, conflicts, and traceability before reading a conclusion.
- Executive narrative second.**

Each narrative statement points back to a Part B exhibit or decision node.
- Forms and outputs last.**

Forms are listed only when the case pack or workflow requires them; no blank forms are added.

Recovered source

Calculated from recovered source

Supplemental synthetic UAT

Not recovered / unresolved

FRONT MATTER

QA Reconciliation and Readiness Summary

Recovered source

36 / 45

Numerator: recovered exhibits. Denominator: all case exhibits.

Calculated

0 / 45

Numerator: calculated exhibits. Denominator: all case exhibits.

Supplemental UAT

8 / 45

Numerator: supplemental UAT exhibits. Denominator: all case exhibits.

Unresolved

1 / 45

Numerator: unresolved exhibits. Denominator: all case exhibits.

Control	Current posture	Required review
Source cutoff	2026-10-08	Verify every displayed record is allowed through this cutoff.
Conflict groups	4	Resolve reliance by affected matter; never smooth conflicts.
Conflicting exhibits	6	Confirm each is visible and linked to its source.
Required workflows	9	Confirm workflow coverage is case-supported.
Artifact disposition	Review required	Human visual and evidentiary review is mandatory before approved_for_uat.

PART B

Controlling Evidence

Build and validate the evidence record before relying on an executive narrative.



PART B - EVIDENCE

B2.1

Source Register

Exhibit	Source / label	Period	Posture	Use
Q3-EX-01	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Q3 / 2026-04-25	Supplemental synthetic UAT	Meeting Control & Packet Readiness: Packet Readiness Confirmation
Q3-EX-02	AGD-Q3-001;PRE-Q3-001;ATT-Q3-001;QUO-Q3-001;MIN-APPR-Q3-001	Q3 / 2026-10-08	Recovered source	Meeting Control: Q3 Meeting Control Record — Agenda, Attendance, Quorum
Q3-EX-03	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Q3 / 2026-02-10	Supplemental synthetic UAT	Board Roster & Composition: Board Roster Change — Community Member Seat
Q3-EX-04	QAPI-Q3-DS-001 §2.1	Q3 / 2026-09-30	Recovered source	Census: Q3 Census Summary — Growth Quarter
Q3-EX-05	AUTHORED DATA-QUALITY OBSERVATION — NOT A SOURCE-RECOVERED RECORD — Q2 CLOSE CENSUS WAS NEVER NORMALIZED	Q3 / 2026-09-30	Not recovered / unresolved	Census: Data-Quality Finding — Q2→Q3 Census Carry-Forward Not Reconciled
Q3-EX-06	QAPI-Q3-DS-001 §3	Q3 / 2026-09-02	Recovered source	Staff: New-Hire Roster & Carry-Forward Disciplinary Notes
Q3-EX-07	QM-Q3-016;QM-Q3-017;QM-Q3-018	Q3 / 2026-09-30	Recovered source	Quality Metrics: Hospitalization Rate — Monthly Trend (Jul/Aug/Sep)
Q3-EX-08	QM-Q3-025;AUD-Q3-CL-010	Q3 / 2026-10-01	Recovered source	Quality Metrics: Hospitalization Rate — Cumulative Q3 Total (NEW PIP TRIGGER)

PART B - EVIDENCE

B2.2

Source Register - continued 2

Exhibit	Source / label	Period	Posture	Use
Q3-EX-09	QM-Q3-026;AUD-Q3-CL-005	Q3 / 2026-10-01	Recovered source	Quality Metrics: Care Coordination Documentation — New Metric (NEW PIP TRIGGER)
Q3-EX-10	QM-Q3-019;QM-Q3-020;QM-Q3-021	Q3 / 2026-09-30	Recovered source	Quality Metrics: Wound Infection Rate — Trend Toward Near-Resolution
Q3-EX-11	QM-Q3-001..015	Q3 / 2026-09-30	Recovered source	Quality Metrics: Five Carry-Forward Metrics — Improving, None at Threshold
Q3-EX-12	AUD-Q3-CL-005	Q3 / 2026-10-01	Recovered source	Feeder Audits: Feeder Audit — Care Coordination Gaps (Signed MOCK-CLIN-0022)
Q3-EX-13	AUD-Q3-CL-010	Q3 / 2026-10-01	Recovered source	Feeder Audits: Feeder Audit — Hospitalization Review ("5 Potentially Preventable")
Q3-EX-14	AUD-Q3-CL-003	Q3 / 2026-09-30	Recovered source	Feeder Audits: Feeder Audit — New-Hire Documentation Timeliness
Q3-EX-15	AUD-Q3-CL-011	Q3 / 2026-10-01	Recovered source	Feeder Audits: Feeder Audit — Chart Review (New Finding, MOCK-CLIN-0003)
Q3-EX-16	AE-Q3-001;RCA-Q3-001	Q3 / 2026-07-11	Recovered source	Adverse Events: AE-Q3-001 — Hospitalization (CHF + Anxiety Decompensation)

PART B - EVIDENCE

B2.3

Source Register - continued 3

Exhibit	Source / label	Period	Posture	Use
Q3-EX-17	AE-Q3-002;RCA-Q3-002;COMP-Q3-010	Q3 / 2026-07-19	Recovered source	Adverse Events: AE-Q3-002 — Hospitalization: Fall + Hip Re-Fracture (Critical)
Q3-EX-18	AE-Q3-007;RCA-Q3-005	Q3 / 2026-09-04	Recovered source	Adverse Events: AE-Q3-007 — Hospitalization: Parkinson's + Fall (RCA Pending)
Q3-EX-19	AE-Q3-005;RCA-Q3-004	Q3 / 2026-08-14	Recovered source	Adverse Events: AE-Q3-005 — Patient Fall, Minor Injury (New-Hire Involved, RCA In Progress)
Q3-EX-20	AE-Q3-004;AE-Q3-008;AE-Q3-009	Q3 / 2026-09-22	Recovered source	Adverse Events: Adverse Events Closed — No Systemic Finding
Q3-EX-21	INF-Q3-001..006	Q3 / 2026-09-30	Recovered source	Infection Surveillance: Infection Surveillance Line List Q3
Q3-EX-22	COMP-Q3-006	Q3 / 2026-09-28	Recovered source	Complaints: COMP-Q3-006 — Repeated Missed Visits by New HHA (Open at Q3 Close)
Q3-EX-23	COMP-Q3-009;GB-Q3-001	Q3 / 2026-09-05	Recovered source	Complaints: COMP-Q3-009 — Family Notification Delay, Escalated to GB Ahead of RCA Completion
Q3-EX-24	COMP-Q3-010	Q3 / 2026-07-20	Recovered source	Complaints: COMP-Q3-010 — Family Communication Failure After Fall/Re-Fracture

PART B - EVIDENCE

B2.4

Source Register - continued 4

Exhibit	Source / label	Period	Posture	Use
Q3-EX-25	COMP-Q3-001;002;003;004;007;008	Q3 / 2026-09-30	Recovered source	Complaints: Other Q3 Complaints — Closed Within Timeliness Standard
Q3-EX-26	PIP-TRIG-Q3-007	Q3 / 2026-10-01	Recovered source	PIP Triggers: PIP-TRIG-Q3-007 — Hospitalization Rate Spike (NEW)
Q3-EX-27	PIP-TRIG-Q3-008	Q3 / 2026-10-01	Recovered source	PIP Triggers: PIP-TRIG-Q3-008 — Care Coordination Documentation Gap (NEW)
Q3-EX-28	PIP-TRIG-Q3-001..006	Q3 / 2026-10-01	Recovered source	PIP Triggers: Six Carry-Forward PIP Triggers — Status Bundle
Q3-EX-29	DISC-TRIG-Q3-001	Q3 / 2026-09-28	Recovered source	Disciplinary Triggers: DISC-TRIG-Q3-001 — Repeated Missed Visits, New Hire (MOCK-CLIN-0031)
Q3-EX-30	DISC-TRIG-Q3-002	Q3 / 2026-10-01	Recovered source	Disciplinary Triggers: DISC-TRIG-Q3-002 — Second Documentation Irregularity After Reinstatement (MOCK-CLIN-0003)
Q3-EX-31	DISC-TRIG-Q3-003	Q3 / 2026-09-04	Recovered source	Disciplinary Triggers: DISC-TRIG-Q3-003 — Fall Risk Assessment Not Documented, New Hire PT (MOCK-CLIN-0030)
Q3-EX-32	DISC-TRIG-Q3-004	Q3 / 2026-09-30	Recovered source	Disciplinary Triggers: DISC-TRIG-Q3-004 — Repeated Late Arrivals, New Hire LVN (MOCK-CLIN-0032)

PART B - EVIDENCE

Source Register - continued 5

Exhibit	Source / label	Period	Posture	Use
Q3-EX-33	DISC-TRIG-Q3-005	Q3 / 2026-10-01	Recovered source	Disciplinary Triggers: DISC-TRIG-Q3-005 — Care Coordination Failure, Same Clinician Who Signed the Systemic Audit
Q3-EX-34	CAP-Q3-001	Q3 / 2026-10-01	Recovered source	Corrective Action Plans: CAP-Q3-001 — Hospitalization Reduction Plan
Q3-EX-35	CAP-Q3-002	Q3 / 2026-10-01	Recovered source	Corrective Action Plans: CAP-Q3-002 — Care Coordination Documentation Standard
Q3-EX-36	CAP-Q3-003;CAP-Q3-004;CAP-Q3-005	Q3 / 2026-10-01	Recovered source	Corrective Action Plans: Three CAPs — Supervision, Competency, Onboarding
Q3-EX-37	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Q3 / 2026-06-01	Supplemental synthetic UAT	Governance — Clinical Manager Transition: Clinical Manager Change Notice
Q3-EX-38	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Q3 / 2026-04-18	Supplemental synthetic UAT	Governance — Scope of Services: Proposed Scope of Services Change
Q3-EX-39	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Q3 / 2026-05-20	Supplemental synthetic UAT	Governance — Budget Authorization: Budget Authorization Request — CAP Resourcing
Q3-EX-40	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Q3 / 2026-06-22	Supplemental synthetic UAT	Governance — Media Incident: Media Incident Notice

PART B - EVIDENCE

B2.6

Source Register - continued 6

Exhibit	Source / label	Period	Posture	Use
Q3-EX-41	AUD-Q3-CO-001..008;AUD-Q3-HR-001..004;AUD-Q3-RM-001..005;AUD-Q3-IT-001..005	Q3 / 2026-10-01	Recovered source	Feeder Audits: Compliance / HR / Risk / IT Audits — All Clean or Improving
Q3-EX-42	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Q3 / 2026-04-02	Supplemental synthetic UAT	Governance — Conflict of Interest (Not This Quarter's Matter): Conflict of Interest Disclosure — Vendor Contract Matter
Q3-EX-43	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Q3 / 2026-03-05	Supplemental synthetic UAT	Governance — Licensure Renewal Status (Distinct From the Scope-Change Matter): Licensure Renewal Status
Q3-EX-44	QAPI-Q4-DS-001 §1 (Year-End Note)	Q4 / 2026-12-31	Recovered source	Out-of-Cutoff: Q4 Year-End Census Preview (OUT OF CUTOFF FOR THIS MEETING)
Q3-EX-45	QAPI-Q3-DS-001 §2.1	Q3 / 2026-09-30	Recovered source	Census: Payer Mix / Diagnosis Distribution / Service Area Detail

PART B - EVIDENCE

Evidence Detail

Q3-EX-01 Packet Readiness Confirmation

SOURCE	POSTURE	AS OF	VALIDATION
SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	2026-04-25	validated

Confirmation record that the quarter's QAPI packet passed the readiness gates required to convene the Board.

- Source integrity checked, meeting control assembled, feeder audits present, required sign-offs complete.
- No unresolved critical data-quality defect blocks convening on this packet.

Q3-EX-02 Q3 Meeting Control Record — Agenda, Attendance, Quorum

SOURCE	POSTURE	AS OF	VALIDATION
AGD-Q3-001;PRE-Q3-001;ATT-Q3-001;QUO-Q3-001;MIN-APPR-Q3-001	Recovered source	2026-10-08	validated

12-section agenda distributed 2026-10-05; pre-read confirmed to all 9 committee members; attendance shows 9 of 9 present; quorum confirmed 9:01 AM; prior (Q2) minutes approved unanimously.

- Agenda AGD-Q3-001 distributed 2026-10-05 (highlights: hospitalization spike, 2 new PIPs, 5 carry-forward PIPs, 5 disciplinary items).
- Attendance ATT-Q3-001: 9 of 9 present; quorum QUO-Q3-001 confirmed 9:01 AM by MOCK-CLIN-0026.
- One of the 9 present is the nominee proposed to fill the vacant community-member seat (see Q3-EX-03) — a nominee is not a seated director until the Board formally seats them.
- Prior minutes (Q2 2026) approved unanimously 2026-10-08.

Q3-EX-03 Board Roster Change — Community Member Seat

SOURCE	POSTURE	AS OF	VALIDATION
SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	2026-02-10	validated

A community-member director's term expired and a replacement candidate was proposed for Board seating.

- Outgoing director: term expired 2026-01-31, no disqualifying findings on file.
- Proposed director: community-member category, background/eligibility screen on file.
- Composition mix (clinical / community / financial) remains compliant if seated as proposed.

PART B - EVIDENCE

Evidence Detail - continued 2

Q3-EX-04 Q3 Census Summary — Growth Quarter

SOURCE	POSTURE	AS OF	VALIDATION
QAPI-Q3-DS-001 §2.1	Recovered source	2026-09-30	validated

Active 142 (Jul 1) → 170 (Sep 30). 41 new SOC, 19 discharged, 4 transferred, 183 episodes tracked. Hospitalizations: 9 patients (MOCK-PT-0019, 0034, 0058, 0076, 0092, 0107, 0124, 0138, 0155). 6 ED visits without hospitalization.

- Active at Jul 1 (carry-forward from Q2): 142. Active at Sep 30: 170.
- Hospitalizations Q3: 9 — patient ids MOCK-PT-0019, 0034, 0058, 0076, 0092, 0107, 0124, 0138, 0155.
- ED visits without hospitalization: 6 — MOCK-PT-0025, 0043, 0067, 0111, 0142, 0162.
- Clinician count grew to 33 (28 carry-forward + 3 new hires this quarter — see Q3-EX-06).

Q3-EX-05 Data-Quality Finding — Q2→Q3 Census Carry-Forward Not Reconciled

SOURCE	POSTURE	AS OF	VALIDATION
AUTHORED DATA-QUALITY OBSERVATION — NOT A SOURCE-RECOVERED RECORD — Q2 CLOSE CENSUS WAS NEVER NORMALIZED	Not recovered / unresolved	2026-09-30	conflicting

Q3 opens at 142 active. Q2's own close-of-quarter active count was never recovered (recorded as null in the normalized Q2 record) — so the 142 carry-forward figure cannot be checked against a known Q2 close, the same unreconciled-boundary pattern already flagged Q1→Q2 (DQ-2026-002).

- Q2 normalized record: activeAtClose = null (no recovered Q2 close-of-quarter census).
- Q3 raw source asserts "142 patients (carry-forward from Q2 close)" without a reconcilable Q2 close figure to check it against.
- This is a continuation of the same denominator-continuity risk already on file for the Q1→Q2 boundary; it should be named as its own finding, not silently assumed resolved.
- Reviewer decision required: accept 142 as the working Q3 open count while flagging the unreconciled boundary, or hold the packet pending confirmation. This exhibit does not, by itself, block convening (see Q3-EX-01 packet-readiness gates).

Q3-EX-06 New-Hire Roster & Carry-Forward Disciplinary Notes

SOURCE	POSTURE	AS OF	VALIDATION
QAPI-Q3-DS-001 §3	Recovered source	2026-09-02	validated

Three new hires onboarded this quarter (MOCK-CLIN-0031 HHA, MOCK-CLIN-0032 LVN, MOCK-CLIN-0033 MSW — plus MOCK-CLIN-0029 RN and MOCK-CLIN-0030 PT onboarded earlier in Q3). Carry-forward disciplinary notes: MOCK-CLIN-0018 documentation timeliness improving (86.9%→91%); MOCK-CLIN-0016 monitoring plan clean; MOCK-CLIN-0003 reinstated Q2 with supervision plan, new finding this quarter (see Q3-EX-15/Q3-EX-30); MOCK-CLIN-0004 clean this quarter.

- MOCK-CLIN-0031 (HHA, new hire): onboarded 2026-08-04; one missed in-service, remediated 2026-09-10.
- MOCK-CLIN-0032 (LVN, new hire): onboarded 2026-08-18; competency complete.
- MOCK-CLIN-0030 (PT, new hire): onboarded 2026-07-14; competency verified.
- MOCK-CLIN-0003: suspended Q1, reinstated Q2 with a supervision plan; a new finding surfaces this quarter (Q3-EX-15, Q3-EX-30).
- Training completion: 32 of 33 staff current (MOCK-CLIN-0031's missed in-service was remediated).

PART B - EVIDENCE

Evidence Detail - continued 3

Q3-EX-07 Hospitalization Rate — Monthly Trend (Jul/Aug/Sep)

SOURCE	POSTURE	AS OF	VALIDATION
QM-Q3-016;QM-Q3-017;QM-Q3-018	Recovered source	2026-09-30	validated

Jul 2.8% (watch, elevated but within ≤3%) → Aug 1.9% (within) → Sep 1.2% (within, improving).

- Jul: 4/142 = 2.8% — status "watch, elevated" (up from 7 hospitalizations Q2).
- Aug: 3/155 = 1.9% — within.
- Sep: 2/163 = 1.2% — within, improving trend.
- Read alongside Q3-EX-08: the improving monthly trend and the cumulative quarterly trigger are not the same signal.

Q3-EX-08 Hospitalization Rate — Cumulative Q3 Total (NEW PIP TRIGGER)

SOURCE	POSTURE	AS OF	VALIDATION
QM-Q3-025;AUD-Q3-CL-010	Recovered source	2026-10-01	conflicting

9 hospitalizations / 170 = 5.3% cumulative for the quarter vs ≤3% quarterly target — flagged as a new PIP trigger despite each individual month reading "within" or "watch" (Q3-EX-07). A favorable-looking monthly trend can mask a quarter-level threshold breach.

- Cumulative Q3: 9/170 = 5.3% vs ≤3% quarterly target — status: NEW PIP TRIGGER.
- The monthly series (Q3-EX-07) reads improving/within by September; the cumulative quarterly rate reads critical. Both are simultaneously true — one is not a correction of the other.
- A Board relying only on the September monthly figure would miss the quarter-level trigger entirely.

Q3-EX-09 Care Coordination Documentation — New Metric (NEW PIP TRIGGER)

SOURCE	POSTURE	AS OF	VALIDATION
QM-Q3-026;AUD-Q3-CL-005	Recovered source	2026-10-01	validated

112/155 = 72.3% vs ≥90% target. New metric introduced this quarter — no prior-quarter baseline exists. Flagged as a new PIP trigger; feeder audit AUD-Q3-CL-005 attributes the gap to delayed multi-discipline communication in 9 patients with 3+ ordered disciplines.

- Q3 average: 112/155 = 72.3% vs ≥90% target — status: NEW PIP TRIGGER.
- No Q1/Q2 baseline exists for this metric; it was introduced this quarter.
- AUD-Q3-CL-005 (Q3-EX-12): 9 patients with 3+ ordered disciplines had delayed multi-discipline communication.
- Linked adverse event: AE-Q3-001 (Q3-EX-17) names a care-coordination failure as a contributing factor in a hospitalization.

PART B - EVIDENCE

Evidence Detail - continued 4

Q3-EX-10 Wound Infection Rate — Trend Toward Near-Resolution

SOURCE	POSTURE	AS OF	VALIDATION
QM-Q3-019;QM-Q3-020;QM-Q3-021	Recovered source	2026-09-30	conflicting

Jul 8.9% → Aug 6.4% → Sep 4.1% (first month at/below the ≤5% target since Q1). Source narrative calls this "PIP resolved pending Q4 confirmation" — but the PIP's own approved sustainability criterion (carried forward from Q1: two consecutive quarters ≤5%) is not yet met by a single month.

- Jul: 4/45 = 8.9% (above). Aug: 3/47 = 6.4% (above). Sep: 2/49 = 4.1% (within, first time since Q1).
- Carry-forward PIP-Q1-006 sustainability criterion (Q1 record): "two consecutive quarters ≤5% in all wound strata."
- One favorable month within a quarter is not the same as one favorable QUARTER, let alone two consecutive quarters.

Q3-EX-11 Five Carry-Forward Metrics — Improving, None at Threshold

SOURCE	POSTURE	AS OF	VALIDATION
QM-Q3-001..015	Recovered source	2026-09-30	validated

OASIS Accuracy 85.9%→88.8% (target ≥90%); Visit Doc Timeliness 91.4%→92.6% (target ≥95%); Med Rec at SOC/ROC 86.7%→88.6% (target ≥95%, up from 79.2% Q1); POC Goal Completeness 87.8%→89.8% (target ≥92%); Missed Visit Rate 3.3%→2.3% (target ≤2%). Every one of the five moved in the right direction across Jul/Aug/Sep and every one remains below (or above, for missed-visit) its approved threshold at Sep close.

- OASIS Accuracy: Jul 85.9%, Aug 87.3%, Sep 88.8% — target ≥90%, still below.
- Visit Doc Timeliness: Jul 91.4%, Aug 92.9%, Sep 92.6% (slight Sep regression attributed to new-hire onboarding) — target ≥95%, still below.
- Med Rec at SOC/ROC: Jul 86.7%, Aug 87.9%, Sep 88.6% — target ≥95%, still below (though substantially improved from the Q1 baseline of 79.2%).
- POC Goal Completeness: Jul 87.8%, Aug 89.2%, Sep 89.8% — target ≥92%, still below.
- Missed Visit Rate: Jul 3.3%, Aug 2.8%, Sep 2.3% — target ≤2%, still above.

Q3-EX-12 Feeder Audit — Care Coordination Gaps (Signed MOCK-CLIN-0022)

SOURCE	POSTURE	AS OF	VALIDATION
AUD-Q3-CL-005	Recovered source	2026-10-01	validated

Signed 2026-10-01 by MOCK-CLIN-0022: 9 patients with 3+ ordered disciplines had delayed multi-discipline communication — the finding underlying the new care-coordination PIP trigger.

- Reviewer/signer: MOCK-CLIN-0022. Status: Complete. Signed: 2026-10-01.
- Key finding: care coordination gaps — 9 patients with 3+ disciplines had delayed communication; identified as a new PIP trigger.
- Read alongside Q3-EX-33: the same clinician ID that signed this systemic finding is separately named in an individual disciplinary trigger this same quarter for a care-coordination failure.

PART B - EVIDENCE

Evidence Detail - continued 5

Q3-EX-13 Feeder Audit — Hospitalization Review ("5 Potentially Preventable")

SOURCE	POSTURE	AS OF	VALIDATION
AUD-Q3-CL-010	Recovered source	2026-10-01	conflicting

The audit asserts 9 hospitalizations Q3, "5 potentially preventable," without itemizing which 5. Cross-checking against the individual RCA records (Q3-EX-17..20 and the closed-no-finding bundle Q3-EX-20a) shows only 3 events with a completed RCA naming a systemic root cause and CAP; 2 RCAs (AE-Q3-005, AE-Q3-007) are still open as of packet close. "5 preventable" is the audit's working characterization, not yet a fully substantiated count.

- Audit finding text: "9 hospitalizations Q3; 5 potentially preventable; new PIP trigger."
- Completed RCA + issued CAP: AE-Q3-001 (care-coordination CAP), AE-Q3-002 (fall-protocol CAP), AE-Q3-003 (monitoring-frequency CAP) — 3 confirmed.
- RCA still open at packet close: AE-Q3-005 (in progress), AE-Q3-007 (pending) — up to 2 more could join the "preventable" count, but are not yet confirmed.
- AE-Q3-004, AE-Q3-008, AE-Q3-009 closed with no systemic finding identified (see Q3-EX-20a) — these should not be counted toward "preventable" without a documented systemic finding.
- The Board should authorize the PIP on the confirmed pattern (the cumulative rate breach, Q3-EX-08) without treating the "5 preventable" figure as a fully closed, itemized fact while 2 RCAs remain open.

Q3-EX-14 Feeder Audit — New-Hire Documentation Timeliness

SOURCE	POSTURE	AS OF	VALIDATION
AUD-Q3-CL-003	Recovered source	2026-09-30	validated

New staff (MOCK-CLIN-0031) had 4 late visit notes in September; coaching initiated. Corroborates the same clinician's separate missed-visit disciplinary trigger (Q3-EX-29).

- Signed 2026-09-30. Finding: MOCK-CLIN-0031 had 4 late visit notes in September; coaching initiated.
- This is the same clinician id named in DISC-TRIG-Q3-001 (Q3-EX-29, missed visits) — two independent quality signals on the same new hire this quarter.

Q3-EX-15 Feeder Audit — Chart Review (New Finding, MOCK-CLIN-0003)

SOURCE	POSTURE	AS OF	VALIDATION
AUD-Q3-CL-011	Recovered source	2026-10-01	validated

Chart review: 3 deficiency rows (down from 6 in Q1), including 1 new finding for MOCK-CLIN-0003 — the reinstated clinician (see Q3-EX-06, Q3-EX-30).

- Signed 2026-10-01. Chart deficiency rows down to 3 (from 6 in Q1) — improving overall pattern.
- 1 new finding: MOCK-CLIN-0003, consistent with the pattern in the disciplinary trigger (Q3-EX-30).

PART B - EVIDENCE

Evidence Detail - continued 6

Q3-EX-16 AE-Q3-001 — Hospitalization (CHF + Anxiety Decompensation)

SOURCE	POSTURE	AS OF	VALIDATION
AE-Q3-001;RCA-Q3-001	Recovered source	2026-07-11	validated

MOCK-PT-0019. High severity. RCA complete: care-coordination CAP issued (CAP-Q3-002 lineage). RCA names MOCK-CLIN-0022 as failing to coordinate multi-discipline care prior to hospitalization.

- Event date 2026-07-11. Severity High. RCA-Q3-001 complete.
- RCA finding: care-coordination CAP issued; multi-discipline communication records missing for 3 patients in July.
- Linked disciplinary trigger: DISC-TRIG-Q3-005 (Q3-EX-33), same clinician id MOCK-CLIN-0022.

Q3-EX-17 AE-Q3-002 — Hospitalization: Fall + Hip Re-Fracture (Critical)

SOURCE	POSTURE	AS OF	VALIDATION
AE-Q3-002;RCA-Q3-002;COMP-Q3-010	Recovered source	2026-07-19	validated

MOCK-PT-0034. Critical severity. RCA complete: fall-risk protocol updated. A linked complaint (COMP-Q3-010) documents a family-communication failure after the event. This is the fall pattern underlying the Board-escalated matter and the media inquiry this quarter.

- Event date 2026-07-19. Severity Critical. RCA-Q3-002 complete: fall-risk protocol updated (CAP-Q3-001 lineage).
- COMP-Q3-010: family not adequately informed following the fall/re-fracture — closed, RCA-linked.
- One of 3 items in the Board escalation record (Q3-EX-40 §12 GB-Q3-001) alongside the hospitalization spike and the second MOCK-CLIN-0003 disciplinary matter.

Q3-EX-18 AE-Q3-007 — Hospitalization: Parkinson's + Fall (RCA Pending)

SOURCE	POSTURE	AS OF	VALIDATION
AE-Q3-007;RCA-Q3-005	Recovered source	2026-09-04	unvalidated

MOCK-PT-0124. High severity. RCA-Q3-005 pending (not yet complete as of packet close). Linked disciplinary trigger for a new-hire PT names a fall-risk-assessment documentation gap.

- Event date 2026-09-04. Severity High. RCA-Q3-005: pending, not yet complete.
- Linked disciplinary trigger: DISC-TRIG-Q3-003 (Q3-EX-31) — fall risk not documented at SOC visit; RCA identifies a direct link to the AE.
- Family notification delay (COMP-Q3-009, Q3-EX-24) is also linked to this event.

PART B - EVIDENCE

Evidence Detail - continued 7

Q3-EX-19 AE-Q3-005 — Patient Fall, Minor Injury (New-Hire Involved, RCA In Progress)

SOURCE	POSTURE	AS OF	VALIDATION
AE-Q3-005;RCA-Q3-004	Recovered source	2026-08-14	unvalidated

MOCK-PT-0092. Medium severity. RCA-Q3-004 in progress — new hire (MOCK-CLIN-0031) involved; supervision review underway. The same clinician id is separately the subject of a missed-visit disciplinary trigger this quarter.

- Event date 2026-08-14. Severity Medium. RCA-Q3-004: in progress, new hire involved, supervision review underway.
- Same clinician id (MOCK-CLIN-0031) as the missed-visit disciplinary trigger DISC-TRIG-Q3-001 (Q3-EX-29) and the documentation-timeliness audit finding (Q3-EX-14).

Q3-EX-20 Adverse Events Closed — No Systemic Finding

SOURCE	POSTURE	AS OF	VALIDATION
AE-Q3-004;AE-Q3-008;AE-Q3-009	Recovered source	2026-09-22	validated

AE-Q3-004 (CHF+AKI hospitalization), AE-Q3-008 (post-op pain ED visit), AE-Q3-009 (Afib hospitalization) — all closed with no RCA required and no systemic root cause identified. Topically part of the same hospitalization-count total, but not evidence supporting a systemic PIP finding.

- AE-Q3-004: closed — rapid deterioration, no systemic finding.
- AE-Q3-008: closed — pain protocol updated (individual, not systemic).
- AE-Q3-009: closed — rapid deterioration, no systemic finding.
- These three should not be cited as support for a systemic care-coordination or hospitalization root-cause finding — they were reviewed and closed without one.

Q3-EX-21 Infection Surveillance Line List Q3

SOURCE	POSTURE	AS OF	VALIDATION
INF-Q3-001..006	Recovered source	2026-09-30	validated

6 infections tracked; 4 resolved, 1 monitoring, 1 in palliative management. None of this quarter's scored matters turn on this list — the wound-infection RATE trend (Q3-EX-10) is the controlling exhibit, not the individual line-list entries.

- INF-Q3-001 (surgical-site wound), INF-Q3-002 (catheter-associated UTI), INF-Q3-003 (wound dehiscence + infection): resolved.
- INF-Q3-004: resolved. INF-Q3-005, INF-Q3-006: ongoing/palliative management, not linked to any Board decision this quarter.

PART B - EVIDENCE

Evidence Detail - continued 8

Q3-EX-22 COMP-Q3-006 — Repeated Missed Visits by New HHA (Open at Q3 Close)

SOURCE	POSTURE	AS OF	VALIDATION
COMP-Q3-006	Recovered source	2026-09-28	validated

Scheduling complaint re: repeated missed visits by MOCK-CLIN-0031. Still open at quarter close — no resolution recorded before the packet was assembled.

- Complaint date 2026-09-28. Status at Q3 close: Open — disciplinary trigger reviewed (DISC-TRIG-Q3-001, Q3-EX-29).
- This complaint remaining open at quarter close is itself a fact the Board should weigh alongside the "coaching + monitoring" disciplinary disposition.

Q3-EX-23 COMP-Q3-009 — Family Notification Delay, Escalated to GB Ahead of RCA Completion

SOURCE	POSTURE	AS OF	VALIDATION
COMP-Q3-009;GB-Q3-001	Recovered source	2026-09-05	conflicting

Family not informed of a Parkinson's-patient fall until the next day; complaint escalated to the Board (GB-Q3-001) — but the underlying event's RCA (RCA-Q3-005, Q3-EX-18) is still pending as of packet close. Escalation for Board awareness is appropriate; treating the RCA's eventual findings as already known is not.

- Complaint date 2026-09-05; resolved-by MOCK-CLIN-0027; 6 days to resolve (outside the 5-day standard).
- Escalated to the Board per GB-Q3-001 (one of 3 escalation items this quarter).
- The linked event's RCA (AE-Q3-007 / RCA-Q3-005) is still pending — the Board can and should take up the communication-timeliness complaint now without prejudging the RCA's eventual clinical findings.

Q3-EX-24 COMP-Q3-010 — Family Communication Failure After Fall/Re-Fracture

SOURCE	POSTURE	AS OF	VALIDATION
COMP-Q3-010	Recovered source	2026-07-20	validated

Family communication failure linked directly to AE-Q3-002 (fall + hip re-fracture). Closed, RCA-linked. Directly relevant to this quarter's media-incident matter (Q3-EX-41) — the same fall pattern is what drew the press inquiry.

- Complaint date 2026-07-20; resolved-by MOCK-CLIN-0028; 3 days to resolve (within standard).
- Status: Closed — RCA linked (to AE-Q3-002 / RCA-Q3-002, Q3-EX-17).

PART B - EVIDENCE

Evidence Detail - continued 9

Q3-EX-25 Other Q3 Complaints — Closed Within Timeliness Standard

SOURCE	POSTURE	AS OF	VALIDATION
COMP-Q3-001;002;003;004;007;008	Recovered source	2026-09-30	validated

Six other Q3 complaints (communication/scheduling) resolved within the 5-day standard or otherwise closed; none escalated to the Board and none required by this quarter's scored matters.

- COMP-Q3-001, 002, 003, 004: closed within 5 days.
- COMP-Q3-007, 008: open at Q3 close (scheduling — late-arrival patterns for new-hire clinicians) but not escalated and not required evidence for any node this case scores.

Q3-EX-26 PIP-TRIG-Q3-007 — Hospitalization Rate Spike (NEW)

SOURCE	POSTURE	AS OF	VALIDATION
PIP-TRIG-Q3-007	Recovered source	2026-10-01	validated

New PIP trigger: 9 hospitalizations Q3 (5.3% cumulative); potentially-preventable pattern identified; care-coordination gap identified as a contributing factor.

- Source records: QM-Q3-025, AUD-Q3-CL-010, AE-Q3-001/002/003/007.
- Recommended action: PIP (CAP-Q3-001 lineage — see Q3-EX-35).

Q3-EX-27 PIP-TRIG-Q3-008 — Care Coordination Documentation Gap (NEW)

SOURCE	POSTURE	AS OF	VALIDATION
PIP-TRIG-Q3-008	Recovered source	2026-10-01	validated

New PIP trigger: care coordination documentation 72.3% vs ≥90% target; multi-discipline communication breakdown in 9 patients.

- Source records: QM-Q3-026, AUD-Q3-CL-005, AE-Q3-001.
- Recommended action: PIP (CAP-Q3-002 lineage — see Q3-EX-36).

PART B - EVIDENCE

Evidence Detail - continued 10

Q3-EX-28 Six Carry-Forward PIP Triggers — Status Bundle

SOURCE	POSTURE	AS OF	VALIDATION
PIP-TRIG-Q3-001..006	Recovered source	2026-10-01	validated

PIP-TRIG-Q3-001 (OASIS Accuracy), -002 (Visit Doc Timeliness), -003 (Med Rec), -004 (POC Goal Completeness), -005 (Missed Visit Rate): all "Active — improving." PIP-TRIG-Q3-006 (Wound Infection): "Near-resolved — Sep at 4.1%," explicitly still pending Q4 confirmation before closure, not yet closure-eligible.

- PIP-TRIG-Q3-001..005 status text: "Active — improving" (each); none reads "resolved" or "closure eligible."
- PIP-TRIG-Q3-006 status text: "Near-resolved — Sep at 4.1%" with explicit finding summary "First month below threshold; PIP pending Q4 confirmation before closure."

Q3-EX-29 DISC-TRIG-Q3-001 — Repeated Missed Visits, New Hire (MOCK-CLIN-0031)

SOURCE	POSTURE	AS OF	VALIDATION
DISC-TRIG-Q3-001	Recovered source	2026-09-28	validated

Medium severity. 3 missed visits in first 8 weeks; scheduling/reporting failure; missed in-service adds context. Disposition: coaching + monitoring plan; may escalate if repeated Q4.

- Source records: COMP-Q3-006, AUD-Q3-CL-007.
- Same clinician id (MOCK-CLIN-0031) is also linked to AE-Q3-005 (fall, RCA in progress) and the documentation-timeliness audit (Q3-EX-14).

Q3-EX-30 DISC-TRIG-Q3-002 — Second Documentation Irregularity After Reinstatement (MOCK-CLIN-0003)

SOURCE	POSTURE	AS OF	VALIDATION
DISC-TRIG-Q3-002	Recovered source	2026-10-01	validated

High severity. Second documentation irregularity since Q2 reinstatement; pattern of chart alterations; existing supervision plan may be insufficient. Referred to HR for a second formal review. This is the restricted personnel matter for executive session this quarter.

- Source records: AUD-Q3-CL-011 (Q3-EX-15), chart-review record Q3-003-C.
- MOCK-CLIN-0003 was suspended in Q1 (documentation-alteration finding), reinstated in Q2 with a supervision plan (Q3-EX-06).
- This is a de-identified clinician reference only (clinicianRef pattern) — no name is carried into any Board-facing exhibit.

PART B - EVIDENCE

Evidence Detail - continued 11

Q3-EX-31 **DISC-TRIG-Q3-003 — Fall Risk Assessment Not Documented, New Hire PT (MOCK-CLIN-0030)**

SOURCE	POSTURE	AS OF	VALIDATION
DISC-TRIG-Q3-003	Recovered source	2026-09-04	validated

High severity. Fall-risk assessment not documented at SOC visit for a Parkinson's patient; RCA identifies a direct link to the subsequent fall (AE-Q3-007). Disposition: mandatory competency re-evaluation; supervision plan (CAP-Q3-004).

- Source records: AE-Q3-007 (Q3-EX-18), RCA-Q3-005 (pending).
- Directly patient-safety-linked — the documentation gap is tied to a specific hospitalization by name in the RCA.

Q3-EX-32 **DISC-TRIG-Q3-004 — Repeated Late Arrivals, New Hire LVN (MOCK-CLIN-0032)**

SOURCE	POSTURE	AS OF	VALIDATION
DISC-TRIG-Q3-004	Recovered source	2026-09-30	validated

Medium severity. 4 late arrivals in September; patient complaint filed (COMP-Q3-008); scheduling not notified. Disposition: performance coaching; formal warning if the pattern continues into Q4.

- Source records: COMP-Q3-008, AUD-Q3-CL-003.
- No patient-safety or RCA linkage — a scheduling/communication issue, lower priority than Q3-EX-31.

Q3-EX-33 **DISC-TRIG-Q3-005 — Care Coordination Failure, Same Clinician Who Signed the Systemic Audit**

SOURCE	POSTURE	AS OF	VALIDATION
DISC-TRIG-Q3-005	Recovered source	2026-10-01	conflicting

High severity. RCA-Q3-001 finds MOCK-CLIN-0022 failed to coordinate multi-discipline care for MOCK-PT-0019 prior to hospitalization; multi-discipline communication records missing for 3 patients in July. MOCK-CLIN-0022 is the SAME clinician id who signed AUD-Q3-CL-005 (Q3-EX-12), the feeder audit that identified the systemic care-coordination gap as a new PIP trigger.

- Disposition: individual corrective plan + system PIP separation documented — i.e., the individual matter and the systemic PIP are handled as two distinct, non-substitutable tracks.
- The overlap does not invalidate the systemic audit finding (Q3-EX-12/Q3-EX-09) — a reviewer's personal involvement in a related incident is a reason to have the finding independently corroborated (it is, via AUD-Q3-CL-005's underlying data and RCA-Q3-001), not a reason to discard the systemic finding outright.

PART B - EVIDENCE

Evidence Detail - continued 12

Q3-EX-34 CAP-Q3-001 — Hospitalization Reduction Plan

SOURCE	POSTURE	AS OF	VALIDATION
CAP-Q3-001	Recovered source	2026-10-01	validated

Enhanced cardiac/COPD monitoring protocols; daily weight tracking for high-risk patients. Owner MOCK-CLIN-0027. Due 2026-11-08. Status: Open.

- Source trigger: PIP-TRIG-Q3-007 (Q3-EX-26). Due date falls in Q4 — resourcing tied to Q3-EX-40 budget request.

Q3-EX-35 CAP-Q3-002 — Care Coordination Documentation Standard

SOURCE	POSTURE	AS OF	VALIDATION
CAP-Q3-002	Recovered source	2026-10-01	validated

Mandatory multi-discipline communication log for patients with 3+ disciplines; weekly Clinical Manager review. Owner MOCK-CLIN-0027. Due 2026-11-08. Status: Open.

- Source trigger: PIP-TRIG-Q3-008 (Q3-EX-27). Resourcing tied to Q3-EX-40 budget request.

Q3-EX-36 Three CAPs — Supervision, Competency, Onboarding

SOURCE	POSTURE	AS OF	VALIDATION
CAP-Q3-003;CAP-Q3-004;CAP-Q3-005	Recovered source	2026-10-01	validated

CAP-Q3-003 (MOCK-CLIN-0003 enhanced supervision — same-day countersign, 90 days; owner MOCK-CLIN-0027; due 2026-12-08). CAP-Q3-004 (MOCK-CLIN-0030 fall-risk competency re-eval + mandatory annual fall-risk training all PT/PTA; owner MOCK-CLIN-0026; due 2026-11-08). CAP-Q3-005 (new-hire onboarding enhancement — documentation-timeliness module + 30-day supervisor audit; owner MOCK-CLIN-0023; due 2026-11-01). All Open.

- CAP-Q3-003 source trigger: DISC-TRIG-Q3-002 (Q3-EX-30). Just opened this quarter — due date is Q4 (2026-12-08).
- CAP-Q3-004 source trigger: DISC-TRIG-Q3-003 (Q3-EX-31).
- CAP-Q3-005 source trigger: AUD-Q3-CL-003 new-hire timeliness finding (Q3-EX-14).

PART B - EVIDENCE

Evidence Detail - continued 13

Q3-EX-37 Clinical Manager Change Notice

SOURCE	POSTURE	AS OF	VALIDATION
SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	2026-06-01	validated

The Clinical Manager role requires backfill following an internal transfer.

- Incumbent Clinical Manager transferred to a non-clinical role effective 2026-06-15.
- Proposed successor holds the required license and clinical-manager qualification on file.
- A coverage gap plan is proposed for the transition period.

Q3-EX-38 Proposed Scope of Services Change

SOURCE	POSTURE	AS OF	VALIDATION
SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	2026-04-18	validated

Management proposes adding a new service line not currently within the agency's licensed scope.

- Proposed addition: a therapy service line not on the current license.
- State licensure amendment and accreditation notification are prerequisites, not follow-up steps.
- No operational start date may precede Board approval and the licensure amendment.

Q3-EX-39 Budget Authorization Request — CAP Resourcing

SOURCE	POSTURE	AS OF	VALIDATION
SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	2026-05-20	validated

Management requests Board authorization of the staffing and system resources the current CAP identified as required to sustain the fix.

- Requested resources: 0.5 FTE quality-review staffing and a documentation-audit tool license.
- The CAP's own effectiveness criteria assume these resources are in place by next quarter close.
- Without authorization, the CAP's effectiveness cannot be sustained per its own stated design.

PART B - EVIDENCE

Evidence Detail - continued 14

Q3-EX-40 Media Incident Notice

SOURCE	POSTURE	AS OF	VALIDATION
SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	2026-06-22	validated

A local news inquiry was received regarding a patient-care incident already under internal review.

- Inquiry received from a local outlet requesting comment within 48 hours.
- Internal RCA on the underlying incident is in progress and not yet complete.
- No confirmed clinical findings should be disclosed publicly ahead of RCA completion.

Q3-EX-41 Compliance / HR / Risk / IT Audits — All Clean or Improving

SOURCE	POSTURE	AS OF	VALIDATION
AUD-Q3-CO-001..008;AUD-Q3-HR-001..004;AUD-Q3-RM-001..005;AUD-Q3-IT-001..005	Recovered source	2026-10-01	validated

Billing/compliance audits improving (doc-to-claim mismatches down, \$0 overpayment identified, policy scorecard 13/14 ≥90%); HR training 32/33 current; risk/safety audit logs 9 adverse events (already covered individually above); IT security clean (2 phishing attempts blocked, 1 access gap resolved same day). None of these audits is required evidence for any scored matter this quarter.

- AUD-Q3-CO-001..008: billing/compliance, all improving or clean.
- AUD-Q3-HR-001..004: training 32/33 complete.
- AUD-Q3-RM-001..005: risk/safety — restates the 9 adverse events/2 falls already itemized individually (Q3-EX-16..20).
- AUD-Q3-IT-001..005: security — 2 phishing attempts blocked, 1 access-control gap resolved same day, HIPAA training current.

Q3-EX-42 Conflict of Interest Disclosure — Vendor Contract Matter

SOURCE	POSTURE	AS OF	VALIDATION
SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	2026-04-02	validated

A director disclosed a financial interest in a vendor whose contract renewal is before the Board this quarter.

- Director holds an equity interest in the vendor entity under consideration for contract renewal.
- Disclosure submitted in advance of the meeting per the conflict-of-interest policy.
- Recusal from deliberation and vote on the vendor matter is the policy-required response.

PART B - EVIDENCE

Evidence Detail - continued 15

Q3-EX-43 Licensure Renewal Status

SOURCE	POSTURE	AS OF	VALIDATION
SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	2026-03-05	validated

The agency's state license renewal is due within the quarter; accreditation renewal follows next quarter.

- State license renewal application status: submitted, decision pending.
- Accreditation renewal survey window opens next quarter.
- No lapse has occurred yet; Board visibility is requested to track the pending decision.

Q3-EX-44 Q4 Year-End Census Preview (OUT OF CUTOFF FOR THIS MEETING)

SOURCE	POSTURE	AS OF	VALIDATION
QAPI-Q4-DS-001 §1 (Year-End Note)	Recovered source	2026-12-31	unvalidated

Year-end census target 200 by Dec 31, 2026 — dated after this Q3 meeting's cutoff (2026-10-08). This exhibit must not be cited to support any Q3 decision; doing so is itself a source-cutoff/evidence-integrity failure regardless of whether the underlying number is accurate.

- Q4 dataset (QAPI-Q4-DS-001) postdates this Q3 meeting.
- Included here only to test whether a decision cites evidence dated after the matter's own cutoff — see ../engine/sourceCutoff.ts.

Q3-EX-45 Payer Mix / Diagnosis Distribution / Service Area Detail

SOURCE	POSTURE	AS OF	VALIDATION
QAPI-Q3-DS-001 §2.1	Recovered source	2026-09-30	validated

71% Medicare A, 18% Medi-Cal, 11% Medicare Advantage. Diagnosis distribution 27% CHF/COPD/Cardiac, 17% Wound, 15% Ortho, 13% Neuro, 9% DM, 8% Onco/Palliative, 6% Psych/MSW, 5% multi-morbidity. Service area split North 26%/South 27%/East 25%/West 22%. Administrative/demographic detail — not required evidence for any scored matter this quarter.

- Payer mix and diagnosis distribution are useful operating context but do not, by themselves, support or refute any of this quarter's PIP, disciplinary, or governance decisions.

PART B - EVIDENCE

Conflict, Gap, and Reliance Register

q3-hospitalization-frame

Monthly hospitalization rates mask the cumulative trigger

Question: Do improving monthly points resolve a quarter-level hospitalization threshold breach?

Why it matters: The Board must not let a favorable presentation frame hide a quarter-level patient-safety trigger.

Affected evidence: Q3-EX-07, Q3-EX-08

Cutoff: 2026-10-08

q3-wound-closure

One qualifying month does not satisfy the PIP criterion

Question: Does September's improvement prove the wound-infection PIP is sustainably resolved?

Why it matters: Closing on one favorable month would replace the Board-approved sustainability test with a weaker standard.

Affected evidence: Q3-EX-10, Q3-EX-28

Cutoff: 2026-10-08

PART B - EVIDENCE

B4.2

Conflict, Gap, and Reliance Register - continued 2

q3-preventable-count

The “five preventable” count is not fully substantiated

Question: May the Board rely on the audit’s working count before all event-level RCAs are complete?

Why it matters: The Board can act on a risk signal without presenting an unfinished working count as a final finding.

Affected evidence: Q3-EX-13, Q3-EX-20

Cutoff: 2026-10-08

q3-complaint-before-rca

The complaint reached the Board before the RCA was complete

Question: What may the Board rely on when the complaint is known but the event findings remain open?

Why it matters: Awareness of a complaint does not authorize the Board to treat unfinished RCA findings as established fact.

Affected evidence: Q3-EX-23, Q3-EX-18

Cutoff: 2026-10-08

PART B - EVIDENCE

B5.1

Decision and Accountability Register

Node	Matter / round	Board question	Evidence	Forms
Q3-N01	M-INTAKE / round 0	Quorum at Convening	Q3-EX-02, Q3-EX-03	GB-FORM-ROSTER-ATTEST
Q3-N02	M-INTAKE / round 0	Public vs. Executive Session	Q3-EX-30	GB-FORM-RESTRICTED-MATTER, GB-FORM-PUBLIC-MINUTES
Q3-N03	M-INTAKE / round 0	Classify the Board Book by Evidentiary Posture	Q3-EX-01, Q3-EX-02	GB-FORM-PACKET-READINESS
Q3-N04	M-HOSP / round 1	Build the Hospitalization Evidence Chain	Q3-EX-08, Q3-EX-13, Q3-EX-26	GB-FORM-PIP-AUTHORIZATION, GB-FORM-RCA-ESCALATION
Q3-N05	M-HOSP / round 1	Authorize the Hospitalization-Reduction PIP	Q3-EX-08, Q3-EX-26	GB-FORM-PIP-AUTHORIZATION
Q3-N06	M-HOSP / round 1	Select the Governing Body Workflows Engaged	Q3-EX-16, Q3-EX-26	GB-FORM-RCA-ESCALATION, GB-FORM-PIP-AUTHORIZATION
Q3-N07	M-CARECOORD / round 2	Authorize the Care-Coordination PIP	Q3-EX-09, Q3-EX-12, Q3-EX-27	GB-FORM-PIP-AUTHORIZATION
Q3-N08	M-CARRYFORWARD / round 3	Reconcile the Wound Infection Closure Question	Q3-EX-10, Q3-EX-28	GB-FORM-PIP-CLOSURE

PART B - EVIDENCE

B5.2

Decision and Accountability Register - continued

2

Node	Matter / round	Board question	Evidence	Forms
Q3-N09	M-CARRYFORWARD / round 3	Disposition of the Five Carry-Forward PIPs	Q3-EX-11, Q3-EX-28	GB-FORM-PIP-CLOSURE, GB-FORM-PIP-AUTHORIZATION
Q3-N10	M-NEWHIRE / round 4	Rank the New-Hire Disciplinary Triggers	Q3-EX-29, Q3-EX-31, Q3-EX-32	GB-FORM-RESTRICTED-MATTER
Q3-N11	M-NEWHIRE / round 4	Assign Owner and Due Date — Fall-Risk Competency CAP	Q3-EX-36	GB-FORM-CAP-EFFECTIVENESS
Q3-N12	M-REINSTATEMENT / round 5	Board vs. Management — The Second Irregularity	Q3-EX-30, Q3-EX-36	GB-FORM-RESTRICTED-MATTER
Q3-N13	M-REINSTATEMENT / round 5	Confidential vs. Public Minutes — The Reinstatement Matter	Q3-EX-30	GB-FORM-EXEC-SESSION-MINUTES, GB-FORM-PUBLIC-MINUTES
Q3-N14	M-REINSTATEMENT / round 5	Effectiveness of the Enhanced Supervision CAP	Q3-EX-36	GB-FORM-CAP-EFFECTIVENESS
Q3-N15	M-CM / round 6	Clinical Manager Transition — Workflow Selection	Q3-EX-37	GB-FORM-CLINICAL-MANAGER-CHANGE
Q3-N16	M-SCOPE / round 6	Scope-of-Services Eligibility	Q3-EX-38	GB-FORM-SCOPE-CHANGE

PART B - EVIDENCE

B5.3

Decision and Accountability Register - continued

3

Node	Matter / round	Board question	Evidence	Forms
Q3-N17	M-BUDGET / round 6	Budget Authorization — CAP Resourcing	Q3-EX-34, Q3-EX-35, Q3-EX-39	GB-FORM-BUDGET-AUTHORIZATION, GB-FORM-CAP-EFFECTIVENESS
Q3-N18	M-MEDIA / round 6	Public Minutes — The Media Inquiry	Q3-EX-40	GB-FORM-MEDIA-INCIDENT, GB-FORM-PUBLIC-MINUTES

PART B - EVIDENCE

Workflow Coverage and Generated Forms

Required workflow coverage

- GV-WF-01
- GV-WF-04
- GV-WF-05
- GV-WF-06
- GV-WF-07
- GV-WF-08
- GV-WF-09
- GV-WF-10
- GV-WF-13

Case-supported forms

- GB-FORM-BUDGET-AUTHORIZATION
- GB-FORM-CAP-EFFECTIVENESS
- GB-FORM-CLINICAL-MANAGER-CHANGE
- GB-FORM-DIRECTOR-APPOINTMENT
- GB-FORM-EXEC-SESSION-MINUTES
- GB-FORM-MEDIA-INCIDENT
- GB-FORM-PACKET-READINESS
- GB-FORM-PIP-AUTHORIZATION
- GB-FORM-PIP-CLOSURE
- GB-FORM-PUBLIC-MINUTES
- GB-FORM-QAPI-PACKET-REVIEW
- GB-FORM-RCA-ESCALATION
- GB-FORM-RESTRICTED-MATTER
- GB-FORM-ROSTER-ATTEST
- GB-FORM-SCOPE-CHANGE
- GB-FORM-TRAINING-ATTESTATION

No blank-form padding

This packet lists only forms referenced by approved case evidence or decision nodes. It does not add blank pages merely to increase packet length.

PART A

Executive Narrative

Every statement below is anchored to a Part B source or decision record.



PART A - EXECUTIVE NARRATIVE

Evidence, Risk, Decisions, and Readiness

Evidence story

Q3-EX-01: Confirmation record that the quarter's QAPI packet passed the readiness gates required to convene the Board. Supplemental synthetic UAT

Q3-EX-02: 12-section agenda distributed 2026-10-05; pre-read confirmed to all 9 committee members; attendance shows 9 of 9 present; quorum confirmed 9:01 AM; prior (Q2) minutes approved unanimously. Recovered source

Q3-EX-03: A community-member director's term expired and a replacement candidate was proposed for Board seating. Supplemental synthetic UAT

Q3-EX-04: Active 142 (Jul 1) → 170 (Sep 30). 41 new SOC, 19 discharged, 4 transferred, 183 episodes tracked. Hospitalizations: 9 patients (MOCK-PT-0019, 0034, 0058, 0076, 0092, 0107, 0124, 0138, 0155). 6 ED visits without hospitalization. Recovered source

Q3-EX-05: Q3 opens at 142 active. Q2's own close-of-quarter active count was never recovered (recorded as null in the normalized Q2 record) — so the 142 carry-forward figure cannot be checked against a known Q2 close, the same unreconciled-boundary pattern already flagged Q1→Q2 (DQ-2026-002). Not recovered / unresolved

Q3-EX-06: Three new hires onboarded this quarter (MOCK-CLIN-0031 HHA, MOCK-CLIN-0032 LVN, MOCK-CLIN-0033 MSW — plus MOCK-CLIN-0029 RN and MOCK-CLIN-0030 PT onboarded earlier in Q3). Carry-forward disciplinary notes: MOCK-CLIN-0018 documentation timeliness improving (86.9%→91%); MOCK-CLIN-0016 monitoring plan clean; MOCK-CLIN-0003 reinstated Q2 with supervision plan, new finding this quarter (see Q3-EX-15/Q3-EX-30); MOCK-CLIN-0004 clean this quarter. Recovered source

Risk story

1 unresolved and 6 conflicting exhibits require explicit reliance judgment. 4 authored conflict groups affect the case.

Decisions and accountability

18 case-supported decision nodes require evidence-linked action. Required workflows: GV-WF-01, GV-WF-04, GV-WF-05, GV-WF-06, GV-WF-07, GV-WF-08, GV-WF-09, GV-WF-10, GV-WF-13.

Reliance determination

This artifact remains REVIEW REQUIRED. It does not claim lock readiness, a completed vote, or official approval.

CONTROL REFERENCES

T1.1

Source-to-Fact Traceability Ledger

Fact / decision	Controlling source	Posture	Purpose
Q3-EX-01	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	Packet Readiness Confirmation
Q3-EX-02	AGD-Q3-001;PRE-Q3-001;ATT-Q3-001;QUO-Q3-001;MIN-APPR-Q3-001	Recovered source	Q3 Meeting Control Record — Agenda, Attendance, Quorum
Q3-EX-03	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	Board Roster Change — Community Member Seat
Q3-EX-04	QAPI-Q3-DS-001 §2.1	Recovered source	Q3 Census Summary — Growth Quarter
Q3-EX-05	AUTHORED DATA-QUALITY OBSERVATION — NOT A SOURCE-RECOVERED RECORD — Q2 CLOSE CENSUS WAS NEVER NORMALIZED	Not recovered / unresolved	Data-Quality Finding — Q2→Q3 Census Carry-Forward Not Reconciled
Q3-EX-06	QAPI-Q3-DS-001 §3	Recovered source	New-Hire Roster & Carry-Forward Disciplinary Notes
Q3-EX-07	QM-Q3-016;QM-Q3-017;QM-Q3-018	Recovered source	Hospitalization Rate — Monthly Trend (Jul/Aug/Sep)
Q3-EX-08	QM-Q3-025;AUD-Q3-CL-010	Recovered source	Hospitalization Rate — Cumulative Q3 Total (NEW PIP TRIGGER)
Q3-EX-09	QM-Q3-026;AUD-Q3-CL-005	Recovered source	Care Coordination Documentation — New Metric (NEW PIP TRIGGER)
Q3-EX-10	QM-Q3-019;QM-Q3-020;QM-Q3-021	Recovered source	Wound Infection Rate — Trend Toward Near-Resolution
Q3-EX-11	QM-Q3-001..015	Recovered source	Five Carry-Forward Metrics — Improving, None at Threshold
Q3-EX-12	AUD-Q3-CL-005	Recovered source	Feeder Audit — Care Coordination Gaps (Signed MOCK-CLIN-0022)

CONTROL REFERENCES

Source-to-Fact Traceability Ledger - continued 2

Fact / decision	Controlling source	Posture	Purpose
Q3-EX-13	AUD-Q3-CL-010	Recovered source	Feeder Audit — Hospitalization Review ("5 Potentially Preventable")
Q3-EX-14	AUD-Q3-CL-003	Recovered source	Feeder Audit — New-Hire Documentation Timeliness
Q3-EX-15	AUD-Q3-CL-011	Recovered source	Feeder Audit — Chart Review (New Finding, MOCK-CLIN-0003)
Q3-EX-16	AE-Q3-001;RCA-Q3-001	Recovered source	AE-Q3-001 — Hospitalization (CHF + Anxiety Decompensation)
Q3-EX-17	AE-Q3-002;RCA-Q3-002;COMP-Q3-010	Recovered source	AE-Q3-002 — Hospitalization: Fall + Hip Re-Fracture (Critical)
Q3-EX-18	AE-Q3-007;RCA-Q3-005	Recovered source	AE-Q3-007 — Hospitalization: Parkinson's + Fall (RCA Pending)
Q3-EX-19	AE-Q3-005;RCA-Q3-004	Recovered source	AE-Q3-005 — Patient Fall, Minor Injury (New-Hire Involved, RCA In Progress)
Q3-EX-20	AE-Q3-004;AE-Q3-008;AE-Q3-009	Recovered source	Adverse Events Closed — No Systemic Finding
Q3-EX-21	INF-Q3-001..006	Recovered source	Infection Surveillance Line List Q3
Q3-EX-22	COMP-Q3-006	Recovered source	COMP-Q3-006 — Repeated Missed Visits by New HHA (Open at Q3 Close)
Q3-EX-23	COMP-Q3-009;GB-Q3-001	Recovered source	COMP-Q3-009 — Family Notification Delay, Escalated to GB Ahead of RCA Completion
Q3-EX-24	COMP-Q3-010	Recovered source	COMP-Q3-010 — Family Communication Failure After Fall/Re-Fracture

CONTROL REFERENCES

T1.3

Source-to-Fact Traceability Ledger - continued 3

Fact / decision	Controlling source	Posture	Purpose
Q3-EX-25	COMP-Q3-001;002;003;004;007;008	Recovered source	Other Q3 Complaints — Closed Within Timeliness Standard
Q3-EX-26	PIP-TRIG-Q3-007	Recovered source	PIP-TRIG-Q3-007 — Hospitalization Rate Spike (NEW)
Q3-EX-27	PIP-TRIG-Q3-008	Recovered source	PIP-TRIG-Q3-008 — Care Coordination Documentation Gap (NEW)
Q3-EX-28	PIP-TRIG-Q3-001..006	Recovered source	Six Carry-Forward PIP Triggers — Status Bundle
Q3-EX-29	DISC-TRIG-Q3-001	Recovered source	DISC-TRIG-Q3-001 — Repeated Missed Visits, New Hire (MOCK-CLIN-0031)
Q3-EX-30	DISC-TRIG-Q3-002	Recovered source	DISC-TRIG-Q3-002 — Second Documentation Irregularity After Reinstatement (MOCK-CLIN-0003)
Q3-EX-31	DISC-TRIG-Q3-003	Recovered source	DISC-TRIG-Q3-003 — Fall Risk Assessment Not Documented, New Hire PT (MOCK-CLIN-0030)
Q3-EX-32	DISC-TRIG-Q3-004	Recovered source	DISC-TRIG-Q3-004 — Repeated Late Arrivals, New Hire LVN (MOCK-CLIN-0032)
Q3-EX-33	DISC-TRIG-Q3-005	Recovered source	DISC-TRIG-Q3-005 — Care Coordination Failure, Same Clinician Who Signed the Systemic Audit
Q3-EX-34	CAP-Q3-001	Recovered source	CAP-Q3-001 — Hospitalization Reduction Plan
Q3-EX-35	CAP-Q3-002	Recovered source	CAP-Q3-002 — Care Coordination Documentation Standard
Q3-EX-36	CAP-Q3-003;CAP-Q3-004;CAP-Q3-005	Recovered source	Three CAPs — Supervision, Competency, Onboarding

CONTROL REFERENCES

Source-to-Fact Traceability Ledger - continued 4

Fact / decision	Controlling source	Posture	Purpose
Q3-EX-37	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	Clinical Manager Change Notice
Q3-EX-38	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	Proposed Scope of Services Change
Q3-EX-39	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	Budget Authorization Request — CAP Resourcing
Q3-EX-40	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	Media Incident Notice
Q3-EX-41	AUD-Q3-CO-001..008;AUD-Q3-HR-001..004;AUD-Q3-RM-001..005;AUD-Q3-IT-001..005	Recovered source	Compliance / HR / Risk / IT Audits — All Clean or Improving
Q3-EX-42	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	Conflict of Interest Disclosure — Vendor Contract Matter
Q3-EX-43	SUPPLEMENTAL SYNTHETIC UAT RECORD — ADDED FOR WORKFLOW COVERAGE — NOT SOURCE-RECOVERED	Supplemental synthetic UAT	Licensure Renewal Status
Q3-EX-44	QAPI-Q4-DS-001 §1 (Year-End Note)	Recovered source	Q4 Year-End Census Preview (OUT OF CUTOFF FOR THIS MEETING)
Q3-EX-45	QAPI-Q3-DS-001 §2.1	Recovered source	Payer Mix / Diagnosis Distribution / Service Area Detail
Q3-N01	Q3-EX-02, Q3-EX-03	Case-pack decision node	Quorum at Convening
Q3-N02	Q3-EX-30	Case-pack decision node	Public vs. Executive Session
Q3-N03	Q3-EX-01, Q3-EX-02	Case-pack decision node	Classify the Board Book by Evidentiary Posture

CONTROL REFERENCES

T1.5

Source-to-Fact Traceability Ledger - continued 5

Fact / decision	Controlling source	Posture	Purpose
Q3-N04	Q3-EX-08, Q3-EX-13, Q3-EX-26	Case-pack decision node	Build the Hospitalization Evidence Chain
Q3-N05	Q3-EX-08, Q3-EX-26	Case-pack decision node	Authorize the Hospitalization-Reduction PIP
Q3-N06	Q3-EX-16, Q3-EX-26	Case-pack decision node	Select the Governing Body Workflows Engaged
Q3-N07	Q3-EX-09, Q3-EX-12, Q3-EX-27	Case-pack decision node	Authorize the Care-Coordination PIP
Q3-N08	Q3-EX-10, Q3-EX-28	Case-pack decision node	Reconcile the Wound Infection Closure Question
Q3-N09	Q3-EX-11, Q3-EX-28	Case-pack decision node	Disposition of the Five Carry-Forward PIPs
Q3-N10	Q3-EX-29, Q3-EX-31, Q3-EX-32	Case-pack decision node	Rank the New-Hire Disciplinary Triggers
Q3-N11	Q3-EX-36	Case-pack decision node	Assign Owner and Due Date — Fall-Risk Competency CAP
Q3-N12	Q3-EX-30, Q3-EX-36	Case-pack decision node	Board vs. Management — The Second Irregularity
Q3-N13	Q3-EX-30	Case-pack decision node	Confidential vs. Public Minutes — The Reinstatement Matter
Q3-N14	Q3-EX-36	Case-pack decision node	Effectiveness of the Enhanced Supervision CAP
Q3-N15	Q3-EX-37	Case-pack decision node	Clinical Manager Transition — Workflow Selection

CONTROL REFERENCES

T1.6

Source-to-Fact Traceability Ledger - continued 6

Fact / decision	Controlling source	Posture	Purpose
Q3-N16	Q3-EX-38	Case-pack decision node	Scope-of-Services Eligibility
Q3-N17	Q3-EX-34, Q3-EX-35, Q3-EX-39	Case-pack decision node	Budget Authorization — CAP Resourcing
Q3-N18	Q3-EX-40	Case-pack decision node	Public Minutes — The Media Inquiry

CONTROL REFERENCES

QA

Human Review and Production/UAT Checklist

- Render and inspect every page for clipping, overlap, blank pages, broken glyphs, and unreadable tables.
- Compare every displayed fact to the normalized case-pack source.
- Confirm searchable text, tagged structure, document title/language, bookmarks, and clickable links.
- Confirm every page carries the synthetic classification.
- Confirm no fake signature, vote, quorum, approval, or completed action is presented.
- Approve for UAT only through an authorized human review; generation alone is insufficient.

Human review required

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